

Started on	Sunday, 12 February 2023, 8:15 PM
State	Finished
Completed on	Sunday, 12 February 2023, 8:15 PM
Time taken	12 secs
Marks	0/62
Grade	0 out of 100

Question 1

Not answered

Marked out of 1

An important risk factor for depression and rapid cycling in bipolar disorder is

Select one:

- ☐ Subclinical hyperparathyroidism
- ☐ Subclinical hypothyroidism
- ☐ Subclinical hypoparathyroidism
- ☐ Subclinical hyperthyroidism
- ☐ Subclinical hypopituitarism

Your answer is incorrect.

Symptoms of hypothyroidism include weakness, fatigue, cold intolerance, weight gain, constipation, and somnolence. Psychiatric symptoms include depression (often misdiagnosed as major depressive disorder and hypothyroidism may be missed), also cognitive dysfunction is noted. Psychosis may be seen in severe cases (myxedema madness). Subclinical hypothyroidism is a risk factor for depression and rapid cycling in bipolar disorder.

The correct answer is: Subclinical hypothyroidism

Question **2**

Not answered

Marked out of 1

The male: female ratio of hypothyroidism is

Select one:

- ☐ 2:3
- ☐ 1:15
- ☐ 1:6
- ☐ 1:3
- ☐ 1:4

Your answer is incorrect.

Hypothyroidism is more common in women than in men (in t he UK, female:male ratio of 6:1).

The correct answer is: 1:6



Question 3

Not answered

Marked out of 1

Hoover's test is a type of physical examination done to identify the possible diagnosis of

Select one:

- ☐ Depression
- ☐ Conversion disorder
- ☐ Hypochondriasis
- ☐ Munchausen syndrome
- ☐ Somatisation disorder

Your answer is incorrect.

Hoover's test is useful to identify patients with conversion disorder, who has paralysis with One or more limbs or one side of the face or body may be affected. Here the patient is unable to raise the affected limb from the couch but is able to raise the unaffected limb against resistance with demonstrable pressing down of the heel on the affected side. Ref: Oxford handbook of psychiatry; 746

The correct answer is: Conversion disorder



Question 4

Not answered

Marked out of 1

The clinical feature that is most suggestive of herpes simplex encephalitis is

Select one:

- ☐ Abrupt onset of confusion
- ☐ Chorea
- ☐ Severe anxiety
- ☐ Myoclonus
- ☐ Dystonia

Your answer is incorrect.

Herpes simplex encephalitis: Fairly abrupt onset of confusion, memory impairment, and often seizures. 70% show psychiatric disturbances including acute confusion, depression and psychosis. Fever is common but not invariable.

The correct answer is: Abrupt onset of confusion



Question 5

Not answered

Marked out of 1

A woman who has had Herpes encephalitis develops severe carbohydrate craving, weight gain and inappropriate sexuality. Which of the following is the most likely diagnosis?

Select one:

- ☐ Diabetes mellitus
- ☐ Kluver-Bucy syndrome
- ☐ Cushing syndrome
- ☐ Prader Willi syndrome
- ☐ Hypothyroidism

Your answer is incorrect.

Herpes simplex encephalitis: HSE is the most common cause of Kluver Bucy syndrome in humans; this is characterized by emotional blunting, hyperphagia, visual agnosia and inappropriate sexual behaviour. This is due to bilateral temporal lobe damage and most often only a partial syndrome is evident. Carbamazepine may be used to control some symptoms.

The correct answer is: Kluver-Bucy syndrome



Question 6

Not answered

Marked out of 1

Which of the following statements about Meige's syndrome is incorrect?

Select one:

- ☐ Primary Meige's is idiopathic
- ☐ It disappears with sleep
- ☐ The movement is present both at rest and when active
- ☐ Oral facial dystonia characterized by repetitive blinking and chin thrusting
- ☐ The patients are more likely to be middle-aged men than women

Your answer is incorrect.

Henri Meige described in 1904 a new type of oral facial dystonia characterized by repetitive blinking and chin thrusting. Some patients have lip pursing or tongue movements and, for a few, the movements spread into the shoulders. Primary Meige's is idiopathic; antipsychotics, levodopa and Lewy body dementia can cause secondary Meige's. At times there is a joint interactive movement between the oral movements and the eye movements. The patients are more likely to be women than men and usually at middle age. There is some variation with stress, but the movement is present both at rest and when active; it disappears with sleep. Patients may chew gum, whistle or touch their face in an effort to lessen the movements (Retrieved from <http://www.blepharospasm.org/meiges-syndrome1.html>).

The correct answer is: The patients are more likely to be middle-aged men than women

Question 7

Not answered

Marked out of 1

Which one of the following is a weak inhibitor of cytochrome P450 enzyme system?

Select one:

- ☐ Sertraline
- ☐ Bupropion
- ☐ Citalopram
- ☐ Paroxetine
- ☐ Fluoxetine

Your answer is incorrect.

Fluoxetine and Paroxetine are potent inhibitors of hepatic CYP enzyme systems (CYP2D6, CYP3A4). Sertraline is a less potent inhibitor but this effect is dose related; at higher doses that are commonly used in practice, sertraline may induce clinically important inhibition of the CYP system. Citalopram is a weak inhibitor. Ref: Maudsley prescribing guidelines, 10th edition; 336

The correct answer is: Citalopram

Question 8

Not answered

Marked out of 1

Some patients intentionally falsify their symptoms and past history and fabricate signs of physical or mental disorder with the primary aim of obtaining medical attention and treatment. Which of the following diagnosis is most appropriate in these circumstances?

Select one:

- ☐ Munchausen syndrome
- ☐ Conversion disorder
- ☐ Hypochondriasis
- ☐ Asperger's syndrome
- ☐ Somatisation disorder

Your answer is incorrect.

Factitious disorder (Munchausen's Syndrome): In factitious disorder patients intentionally falsify their symptoms and past history and fabricate signs of physical or mental disorder with the primary aim of obtaining medical attention and treatment. The diagnostic features are the intentional and conscious production of signs, falsification, or exaggeration of the history and the lack of gain beyond medical attention and treatment.

The correct answer is: Munchausen syndrome

Question 9

Not answered

Marked out of 1

Which of the following is not a risk factor associated with depression in Parkinson's disease?

Select one:

- ☐ Slow progression of the disease
- ☐ Bradykinesia and gait disturbance
- ☐ Prominent right-sided signs
- ☐ Female gender
- ☐ Younger onset

Your answer is incorrect.

Depression in Parkinson's disease: Risk factors include Female sex, younger onset, prominent right-sided signs, bradykinesia and gait disturbance, rapid disease progression, poorer cognitive status and activities of daily living. Apathy without depression is also common and is associated with executive dysfunction. It may be masked by Parkinsonian affect - subjective exploration may be necessary.

The correct answer is: Slow progression of the disease



Question 10

Not answered

Marked out of 1

All are true about delirium in palliative care except

Select one:

- ☐ Occurs in over 50% of patients at some stage in their illness
- ☐ May be irreversible
- ☐ May present with sedation
- ☐ Multifactorial in origin
- ☐ Always responds to neuroleptics

Your answer is incorrect.

Delirium is common in palliative care settings. The prevalence is around 44% in cancer in-patients, rising to 62% shortly before death. It may be either hypoactive sedated subtype or hyperactive agitated type. The aetiology is usually multi-factorial, and a specific aetiology is discovered in less than 50% of terminally ill patients with delirium. Though small dose of haloperidol or lorazepam is frequently used, they are not always successful in controlling the symptoms, especially in sedated subtype.

The correct answer is: Always responds to neuroleptics

Question 11

Not answered

Marked out of 1

Poor prognostic factors with respect to psychiatric morbidity following head injury include all except

Select one:

- ☐ Long period of post traumatic amnesia
- ☐ Long duration of loss of consciousness
- ☐ Older age
- ☐ Focal damage restricted to the non-dominant lobe
- ☐ Chronic alcohol use

Your answer is incorrect.

After a head injury, patients who struggle to return to previous activities generally report a longer duration of post-traumatic amnesia and loss of consciousness than those who resume work without much difficulties. Focal non-dominant lobe damage does not indicate poor prognosis.

The correct answer is: Focal damage restricted to the non-dominant lobe

Question 12

Not answered

Marked out of 1

Mr.Y suffers from psychiatric symptoms such as apathy, anhedonia, fatigue, and depressed mood. He reports repeated vomiting. On examination you discover unexplained dark skin pigmentation. This should suggest

Select one:

- ☐ Hypothyroidism
- ☐ Pheochromocytoma
- ☐ Addison's disease
- ☐ Cushing's syndrome
- ☐ Acromegaly

Your answer is incorrect.

Addison's disease: (Cortisol depletion) Psychiatric symptoms - apathy, anhedonia, fatigue, and depressed mood. May be misdiagnosed as major depressive disorder. Anorexia is common in both, but the presence of nausea, vomiting and skin changes (dark pigmentation in some

The correct answer is: Addison's disease

Question 13

Not answered

Marked out of 1

A proportion of patients who are at a high risk of HIV are seronegative but remain anxious about contracting HIV. Which of the following descriptions/diagnosis suits them best?

Select one:

- ☐ Acute stress reaction
- ☐ Hypochondriasis
- ☐ Worried well
- ☐ Worried ill
- ☐ Adjustment disorder

Your answer is incorrect.

The term "worried well" refers to someone who harbours a fear that he or she has the disease or may contract the infection imminently. Some of the characteristics of the worried well include:

- Several negative HIV tests
- Poor sexual adjustment and social isolation
- Multiple somatic features wrongly interpreted as a part of an undiagnosed HIV illness
- Psychiatric history - especially anxiety or depression

Ref: Chippindale & French., BMJ. 2001 Jun 23; 322(7301): 1533-1535.

<http://www.ncbi.nlm.nih.gov/pmc/articles/PMC1120575/>

The correct answer is: Worried well

Question 14

Not answered

Marked out of 1

Depression is thought to be the most common mental illness in those with HIV infection. Which of the following is least likely to explain the high rates of depression noted in HIV-infected individuals?

Select one:

- ☐ Shared risk factors in the two groups
- ☐ Depressive effect of antiretroviral therapy
- ☐ Direct neurotoxicity
- ☐ Spuriously high diagnosis due to shared symptomatology
- ☐ Psychological impact of diagnosis

Your answer is incorrect.

The risk factors for depression are similar to the risk factors for those without HIV. These include; A. advanced HIV infection; B. loss of social support; C. personal and family history of depression; D. drug use; and E. lack of confidants. Poor adherence to medical treatment for HIV can occur as a result of depression, complicating management. Antiretroviral therapy is not a common cause of depression.

The correct answer is: Depressive effect of antiretroviral therapy

Question 15

Not answered

Marked out of 1

Which of the following medications has the best evidence for use in HIV dementia complex?

Select one:

- ☐ Didanosine
- ☐ Memantine
- ☐ Efavirenz
- ☐ Donepezil
- ☐ Zidovudine

Your answer is incorrect.

Patients with HIV dementia may stabilize or improve slightly on antiretroviral therapy (zidovudine has the best evidence in this regard).

The correct answer is: Zidovudine



Question **16**

Not answered

Marked out of 1

Which of the following is a primary organic disorder due to HIV infection?

Select one:

- ☐ Adjustment disorder
- ☐ HIV induced CNS lymphoma
- ☐ HIV dementia
- ☐ Cerebral tuberculosis
- ☐ Cerebral toxoplasmosis

Your answer is incorrect.

Neuropsychiatric disorders in HIV can be either primary organic (directly due to viral CNS damage) e.g. HIV dementia or secondary organic (due to opportunistic infections or drugs) e.g. cerebral toxoplasmosis or reactive e.g. HIV associated acute stress reaction. Prevalance of any mental disorder in the lifetime of HIV positive patients is 38 to 73%.

The correct answer is: HIV dementia

Question 17

Not answered

Marked out of 1

The most frequent cause of psychiatric hospitalisation in those with HIV seropositivity is

Select one:

- ☐ Anxiety
- ☐ Psychosis
- ☐ Depression
- ☐ Mania
- ☐ Dementia

Your answer is incorrect.

HIV seems to increase the risk of manic episodes, and mania is the most frequent reason for psychiatric hospitalization among people with HIV, followed by depression and psychosis.

The correct answer is: Mania



Question **18**

Not answered

Marked out of 1

A 34 year old homosexual man presents with forgetfulness, loss of concentration, mental slowing. He admits to unsafe sexual practices. He appears clumsy and apathetic. Which of the following statements is false with regard to his diagnosis?

Select one:

- ☐ Laboratory evidence of HIV infection is necessary to make the diagnosis
- ☐ Aphasia, agnosia and apraxia are unusual
- ☐ Decline in motor function may be present
- ☐ The minimum duration of symptoms must be 1 year to make the diagnosis
- ☐ Decline in memory may not be severe enough to impair all ADLs

Your answer is incorrect.

Dementia is noted concurrently with initial AIDS-defining illness in 3% of cases. 1-month duration is sufficient

The correct answer is: The minimum duration of symptoms must be 1 year to make the diagnosis



Question 19

Not answered

Marked out of 1

The most common neuropsychiatric symptom associated with Parkinson's disease is

Select one:

- ☐ Hallucinations
- ☐ Delusions
- ☐ Cognitive impairment
- ☐ Depression
- ☐ Hypomania

Your answer is incorrect.

While depression may be partly reactive to receiving the diagnosis or experiencing a worsening of PD symptoms, the depressive illness may be due to the actual disease process itself. Depression in PD is very frequent (50%) and is more common in women and is often atypical in nature.

The correct answer is: Depression



Question **20**

Not answered

Marked out of 1

In patients with transient global amnesia, the amnesic episode usually resolves within

Select one:

- ☐ 7 days
- ☐ 6 hours
- ☐ 1 hour
- ☐ 24 hours
- ☐ 3 days

Your answer is incorrect.

Transient global amnesia is characterized by the abrupt onset anterograde amnesia characterized by significant new learning deficit" (Excerpt from Oxford Handbook of Psychiatry). Patients show mild confusion and a lack of insight into the problem but an intact sensorium. Episodes last from 6 to 24 hours. Though the aetiology is unclear, most cases involve hypoperfusion in the temporal and parietotemporal regions, particularly in the left hemisphere. Most patients show complete improvement though recurrence is high.

The correct answer is: 24 hours



Question **21**

Not answered

Marked out of 1

Which of the following treatments for MS is most likely to lead to depression?

Select one:

- ☐ Baclofen
- ☐ Steroids
- ☐ Glatiramer acetate
- ☐ Beta-interferon
- ☐ Amantadine

Your answer is incorrect.

Though controversial, only a weak association has been reported with beta interferon. Note that among the various interferons, it is Interferon alpha which is clearly associated with depression (used in viral hepatitis).

The correct answer is: Steroids



Question **22**

Not answered

Marked out of 1

In post-stroke depression, the antidepressant treatment should be continued for at least

Select one:

- ☐ Two months
- ☐ Four months
- ☐ Two years
- ☐ Six months
- ☐ One year

Your answer is incorrect.

There is limited evidence regarding the efficacy of antidepressants in treating post-stroke depression. Although there is the suggestion that Tricyclic antidepressants are more effective, SSRIs are a reasonable first choice. Consider a trial of antidepressant medication in those with a persistently depressed mood of at least 1-month duration. If there is a good response, antidepressants should be continued for at least 6 months.

The correct answer is: Six months



Question **23**

Not answered

Marked out of 1

What is the mean duration of a major depressive episode that follows a stroke?

Select one:

- ☐ 14 Weeks
- ☐ 34 Weeks
- ☐ 4 Weeks
- ☐ 24 Weeks
- ☐ 8 Weeks

Your answer is incorrect.

The mean duration of post-stroke, major depression is 34 weeks

The correct answer is: 34 Weeks



Question **24**

Not answered

Marked out of 1

A 42-year-old woman presents with a history of 2 episodes of unilateral blindness that lasted for few weeks. Now she has paraesthesia of left leg and appears quite depressed. Her MRI brain shows multiple demyelinating plaques. With respect to her depression which of the following is NOT correct?

Select one:

- ☐ Can be treated with SSRIs
- ☐ ECT is absolutely contraindicated
- ☐ Has a lifetime prevalence of up to 50% in those with the same neurological condition
- ☐ Can be treated with cognitive behavioural therapy
- ☐ Can be caused by the drugs used to treat the neurological condition

Your answer is incorrect.

The description is suggestive of multiple sclerosis. A 20% risk of triggering a relapse of multiple sclerosis has been reported if ECT is given. The presence of active brain lesions on MRI before treatment is a potential risk factor for MS relapse following ECT. But MS is not an absolute contraindication for ECT administration.

The correct answer is: ECT is absolutely contraindicated



Question **25**

Not answered

Marked out of 1

Which one of the following cognitive faculties remains intact in amnesic disorders?

Select one:

- ☐ Anterograde memory
- ☐ Retrograde memory
- ☐ Recent memory
- ☐ Immediate memory
- ☐ Short term memory

Your answer is incorrect.

The Amnesic disorders are syndromes characterised by the development of a memory disorder characterized by impairment in the ability to learn new information (anterograde amnesia) and the inability to recall previously remembered knowledge (retrograde amnesia). A general medical condition or use substance usually causes it, and where delirium and dementia have been excluded as causative of the amnesia. In amnesic disorders, short-term and recent memory is usually impaired. Memory for events from the remote past, such as childhood experiences, is generally retained, but the memory for events from the less remote past (over the past decade) is impaired.

The correct answer is: Immediate memory



Question 26

Not answered

Marked out of 1

Which of the following is true concerning cognitive impairment seen in multiple sclerosis?

Select one:

- ☐ Is no more common than in the general population
- ☐ Can be explained by the increased prevalence of depression in multiple sclerosis
- ☐ Can be treated with Donepezil
- ☐ Has the same feature as a subcortical dementia
- ☐ Is best screened for using the Mini-Mental State Examination

Your answer is incorrect.

Subcortical dementia is "a clinical syndrome characterized by slowness of mental processing, forgetfulness, impaired cognition, apathy, and depression. First recognized in progressive supranuclear palsy and Huntington's disease, the concept has been extended to account for the intellectual impairment of Parkinson's disease, Wilson's disease, spinocerebellar degenerations, idiopathic basal ganglia calcification, the lacunar state, and the dementia syndrome of MS"(Excerpt from Cummings, J. L., & Benson, D. F. (1984). Subcortical dementia: Review of an emerging concept. Archives of Neurology, 41(8), 874-879.).

The correct answer is: Has the same feature as a subcortical dementia

Question **27**

Not answered

Marked out of 1

With respect to management of Parkinson's disease which of the following is correct?

Select one:

- ☐ Dementia can be treated with any Acetylcholinesterase inhibitors as RCT evidence is abundant
- ☐ Hypomania is a side effect of levodopa
- ☐ Clozapine must be avoided in Parkinsons psychosis
- ☐ Psychosis is associated with decreases in antiparkinsonian medications
- ☐ Quetiapine is best avoided among all antipsychotics

Your answer is incorrect.

Psychosis in Parkinson's disease is mostly drug related with both dopaminergic/anticholinergic medications being implicated. Hallucinations are common (20%) especially in visual modality.

The correct answer is: Hypomania is a side effect of levodopa



Question **28**

Not answered

Marked out of 1

The frequency of depressive symptoms in epilepsy is

Select one:

- ☐ 1-2%
- ☐ 0.75
- ☐ 30-50%
- ☐ 0.2
- ☐ 3-7%

Your answer is incorrect.

Psychiatric symptoms in epilepsy: Depression- 30-50%, Panic disorder- 20%, Psychosis- 3-7%

The correct answer is: 30-50%



Question 29

Not answered

Marked out of 1

A 42-year-old man suffering from poorly controlled epilepsy is referred by the neurologist as non-epileptic seizures are suspected. Which of the following statement is correct?

Select one:

- ☐ Non epileptic seizures are more common in males
- ☐ Prolactin levels are elevated after non-epileptic attack
- ☐ Video recording of ictal and periictal behaviour can help in diagnosis
- ☐ Known epileptic patients do not have dissociative seizures
- ☐ Presence of depression suggests non-epileptic attacks in most epileptics.

Your answer is incorrect.

Pseudoseizures (Non epileptic seizures) are often associated with past psychiatric history. Pseudoseizures more likely to happen in the daytime and when others are present. Increased post-ictal serum prolactin concentrations (greater than 1000 IU/l) despite normal baseline levels are found after epileptic seizures but measurements must be taken within 15 min of the event. Video recording along with EEG is the best diagnostic measure.

The correct answer is: Video recording of ictal and periictal behaviour can help in diagnosis

Question **30**

Not answered

Marked out of 1

Which one of the following drugs is particularly epileptogenic and should be used with caution in epileptic psychosis?

Select one:

- ☐ Sulpiride
- ☐ Haloperidol
- ☐ Amisulpride
- ☐ SSRIs
- ☐ Clozapine

Your answer is incorrect.

Prevalence of psychosis in epilepsy is between 3 and 7%. It is more common in those with partial epilepsies. Haloperidol and Sulpiride may be less epileptogenic than other antipsychotics and, therefore, the treatment of choice for epileptic psychosis. Clozapine can worsen epileptic psychosis by lowering seizure threshold.

The correct answer is: Clozapine

Question **31**

Not answered

Marked out of 1

Post-traumatic epilepsy is seen in what proportion of patients with open head injury?

Select one:

- ☐ 30%
- ☐ 1%
- ☐ 50%
- ☐ 10%
- ☐ 5%

Your answer is incorrect.

Post-traumatic epilepsy is seen in 5% closed and 30% open head injuries (usually during first year) and worsens prognosis. Less psychopathology in children after head injury due to increased brain plasticity.

The correct answer is: 30%



Question **32**

Not answered

Marked out of 1

Post-concussional syndrome is characterized by all except;

Select one:

- ☐ Headache
- ☐ Emotional lability
- ☐ Dizziness
- ☐ Poor concentration
- ☐ Passivity symptoms

Your answer is incorrect.

Post-concussional syndrome is characterized by headache, dizziness, fatigue, poor memory, poor concentration, irritability, depression, sleep disturbance, frustration, restlessness, sensitivity to noise, blurred vision, double vision, photophobia, nausea and tinnitus (King, 1997). At least 50% experience some post-concussion symptoms that recover completely within 3 months of injury, except in nearly a third.

The correct answer is: Passivity symptoms

Question **33**

Not answered

Marked out of 1

Which of the following is false about mania seen in multiple sclerosis?

Select one:

- ☐ Is more common than in the general population
- ☐ Predominant frontal distribution of plaques in MRI
- ☐ Can be induced by prescribed treatments for multiple sclerosis
- ☐ Can occur as part of the primary physical disorder.
- ☐ Is treated with mood stabilisers

Your answer is incorrect.

People with multiple sclerosis who become hypomanic on steroid therapy are more likely to have a family or premorbid history of affective disorder and/or alcoholism. There is MRI evidence suggesting that patients showing mania with psychotic symptoms have plaques that are distributed predominantly in the bilateral temporal horn areas

The correct answer is: Predominant frontal distribution of plaques in MRI

Question **34**

Not answered

Marked out of 1

A 45-year-old gentleman was admitted to the medical unit with history of stroke. A psychiatric opinion was requested due to his deteriorating mental state. His MRI brain scan showed lesions proximal to the left frontal lobe. Which one of the following psychiatric conditions is most likely to occur in this scenario?

Select one:

- ☐ Post traumatic stress disorder
- ☐ Depression
- ☐ Psychosis
- ☐ Mania
- ☐ Anxiety disorder

Your answer is incorrect.

According to a meta-analytic study by Narushima et, al. 2003, the proximity of the lesions to the left frontal lobe predicts depressive illness. This is also supported by another study from Finland (Vataja et al. 2004).

The correct answer is: Depression



Question 35

Not answered

Marked out of 1

Which of the following statements about mania in multiple sclerosis is incorrect?

Select one:

- ☐ There is MRI evidence suggesting that patients showing mania with psychotic symptoms have plaques that are distributed predominantly in the bilateral temporal horn areas
- ☐ Mild to moderate degrees of mania seen in up to 2/3rd of patients given steroids.
- ☐ Can also be drug induced
- ☐ Higher than expected prevalence of mania
- ☐ People with multiple sclerosis who become hypomanic on steroid therapy are more likely to have a family history of affective disorder.

Your answer is incorrect.

Mania in multiple sclerosis: Higher than expected prevalence of mania. It can also be drug induced: steroids (more likely), baclofen, dantrolene, tizanidine (central muscle relaxant). Mild to moderate degrees of mania seen in up to 1/3rd of patients given steroids. People with multiple sclerosis who become hypomanic on steroid therapy are more likely to have a family or premorbid history of affective disorder and/or alcoholism. There is MRI evidence suggesting that patients showing mania with psychotic symptoms have plaques that are distributed predominantly in the bilateral temporal horn areas

The correct answer is: Mild to moderate degrees of mania seen in up to 2/3rd of patients given steroids.

Question **36**

Not answered

Marked out of 1

A psychiatric trainee is keen to obtain prolactin concentrations from a patient to determine possibility of a true seizure activity. What is the time limit after a seizure within which the measurements must be taken?

Select one:

- ☐ 15 hours
- ☐ 1 hour
- ☐ 1 min
- ☐ 15 days
- ☐ 15 minutes

Your answer is incorrect.

Increased post-ictal serum prolactin concentrations (greater than 1000 IU/l) despite normal baseline levels are found after epileptic seizures but measurements must be taken within 15 min of the event.

The correct answer is: 15 minutes



Question **37**

Not answered

Marked out of 1

What is the lifetime prevalence of depressive symptoms in patients with multiple sclerosis?

Select one:

- ☐ 4-5%
- ☐ 1-2%
- ☐ 0.25
- ☐ 12-15%
- ☐ 40-50%

Your answer is incorrect.

In patients with multiple sclerosis, lifetime prevalence of depressive symptoms is reported in 40-50%. It is about three times higher than the rate in the general population. Desipramine and SSRIs have an evidence base for their use in the treatment. ECT may be used. However, there is a 20% risk of triggering a relapse of multiple sclerosis if ECT is given

The correct answer is: 40-50%

Question **38**

Not answered

Marked out of 1

A 65-year-old gentleman with Parkinson's disease presented with psychotic symptoms characterized by paranoia and visual hallucinations. His wife does not want his antiparkinsonian medication to be stopped as it has worsened the motor features of Parkinson's disease in the past. Which antipsychotic can improve his symptoms without worsening Parkinsonism?

Select one:

- ☐ Clozapine
- ☐ Sulpiride
- ☐ Haloperidol
- ☐ Trifluoperazine
- ☐ Risperidone

Your answer is incorrect.

In parkinsonian psychosis, consideration should be given to decreasing or stopping antiparkinsonian drugs. Due to the propensity to cause EPSEs antipsychotic medications should be used with caution. There is good evidence that low dose clozapine (less than 100 mg/day) can improve psychosis without worsening parkinsonism (Parkinson study group 1999).

The correct answer is: Clozapine

Question 39

Not answered

Marked out of 1

The gene for Huntington's disease (HD) is located on which chromosome?

Select one:

- ☐ long arm of chromosome four
- ☐ short arm of chromosome six
- ☐ short arm of chromosome five
- ☐ long arm of chromosome four
- ☐ short arm of chromosome four

Your answer is incorrect.

Huntington's disease can manifest at any time between infancy and senescence. The mutant protein in Huntington's disease—huntingtin—results from an expanded CAG repeat leading to a polyglutamine strand of variable length at the N-terminus. This tail confers a toxic gain of function. The gene for Huntington's disease (HD) is located on the short arm of chromosome four and is associated with an expanded trinucleotide repeat.

The correct answer is: short arm of chromosome four

Question 40

Not answered

Marked out of 1

Head injury may predispose to the development of

Select one:

- ☐ Alzheimer's disease
- ☐ Chronic fatigue syndrome
- ☐ Hebephrenic schizophrenia
- ☐ Bipolar affective disorder
- ☐ Conduct disorder

Your answer is incorrect.

Head injuries, particularly in men, may predispose to the development of Alzheimers disease. (Fleminger et al 2003). It is possibly due to deposition of beta amyloid in the brain at the time of the injury, although evidence that it is linked to APOE status is inconsistent. Head injury has long been considered as a possible risk factor for schizophrenia although a comprehensive critical review of psychosis following head injury concludes that the injury is unlikely to be a causal factor in the disorder (David and Prince 2005; Advances in neuropsychiatry: clinical implications | BJPsych. Retrieved from <http://apt.rcpsych.org/content/12/1/23>). Nevertheless, 'psychosis' is known to occur after head injuries.

The correct answer is: Alzheimer's disease

Question **41**

Not answered

Marked out of 1

Mr. Y has a history of epileptic psychosis and remains psychotic. The most appropriate drug of choice is;

Select one:

- ☐ Clozapine
- ☐ Zuclopenthixol
- ☐ Chlorpromazine
- ☐ Haloperidol
- ☐ Depixol

Your answer is incorrect.

Drug-induced low mood is an important differential diagnosis

The correct answer is: Haloperidol



Question **42**

Not answered

Marked out of 1

The psychiatric syndrome most commonly associated with stroke is

Select one:

- ☐ Mania
- ☐ Anxiety disorder
- ☐ Depression
- ☐ Cognitive impairment
- ☐ Psychosis

Your answer is incorrect.

Depressive illness after stroke is extremely common, occurring in up to 50-60% of cases. Its onset is usually between 3 and 24 months following the stroke.

The correct answer is: Depression



Question **43**

Not answered

Marked out of 1

Mrs. Z has a history of epilepsy and has now been diagnosed with depression. Which class of drugs should be used as first a line treatment in her case?

Select one:

- ☐ Antipsychotics
- ☐ Lithium
- ☐ MAOIs
- ☐ SSRI
- ☐ TCAs

Your answer is incorrect.

Depression in epilepsy is mainly associated with the location of lesion - esp. common in TLE. High risk of suicide is present in epilepsy even without depression; mortality shoots when clinical depression is positive (nearly 25 times higher). SSRIs are the first line; may reduce seizure threshold but rare. Lithium can increase seizure severity and reduces the seizure threshold.

The correct answer is: SSRI



Question **44**

Not answered

Marked out of 1

Lesions caused by an injury to which part of the brain predicts depression?

Select one:

- ☐ Left temporal lobe
- ☐ Right temporal lobe
- ☐ Right parietal lobe
- ☐ Right frontal lobe
- ☐ Left frontal lobe

Your answer is incorrect.

Depression (most common sequelae) and anxiety occur in roughly 1/4 of head injury survivors. Meta-analysis shows that proximity of lesion to left frontal lobe predicts depression. Suicide risk is also higher post head injury. A schizophrenia-like psychosis with prominent paranoia is associated with left temporal injury, while affective psychoses (esp. mania in 9% patients) are associated with right temporal or orbitofrontal injury. There is also an increased prevalence of schizophrenia post head injury (-2.5% develop the disorder).

The correct answer is: Left frontal lobe

Question 45

Not answered

Marked out of 1

Which of the following is a risk factor associated with cognitive impairment in Parkinson's disease?

Select one:

- ☐ Low education
- ☐ Younger patients
- ☐ Early -onset Parkinson's disease
- ☐ High socio-economic status
- ☐ Less extrapyramidal signs

Your answer is incorrect.

Cognitive impairment in Parkinson's disease: It is Common; poor prognosis if present at time of initial referral. Ranges from mild to severe dementia syndrome. Risk factors are Older patients, late-onset Parkinson's disease, low socio-economic status, low education and severe extrapyramidal signs. Lewy body dementia is closely linked pathologically; rivastigmine is useful in LBD. No drugs licensed yet for Parkinson related dementia. Neuroleptic sensitivity is seen. Higher incidence of hallucination and delusions than Alzheimer's. Frontal executive dysfunction - a subcortical pattern is seen.

The correct answer is: Low education



Question 46

Not answered

Marked out of 1

Concerning movement disorders in Parkinson's disease all are true except

Select one:

- ☐ Medication may affect psychiatric symptoms
- ☐ They may mask depression
- ☐ They may be aggravated by psychotropic medication
- ☐ Levodopa causes dose dependent psychiatric manifestations
- ☐ Presence of cognitive impairment improves prognosis

Your answer is incorrect.

Almost all patients with PD suffer from selective cognitive impairments, including difficulties with attention, concentration, problem solving, set-shifting, and memory, which are thought to reflect dysfunction of cortical circuits subserving frontal brain regions. These impairments are most frequently reported by patients in terms of the disabilities they cause, such as difficulties in paying attention at work; problems handling more than one project at a time; inability to sequence, plan, and organize tasks at work and home; and problems completing tasks that have been started. These disabilities present serious complications for patients in the management of their everyday lives, particularly with regard to occupational functioning(Cognitive Impairment in Parkinson's Disease - Primary Psychiatry. Retrieved from <http://primarypsychiatry.com/cognitive-impairment-in-parkinsonas-disease/>).

The correct answer is: Presence of cognitive impairment improves prognosis

Question **47**

Not answered

Marked out of 1

Huntington's disease is transmitted through which of the following genetic changes?

Select one:

- ☐ Autosomal recessive gene on the short arm of chromosome 4
- ☐ Autosomal dominant gene on the short arm of chromosome 4
- ☐ Autosomal recessive gene on the short arm of chromosome 4
- ☐ Autosomal dominant gene on the long arm of chromosome 4
- ☐ X-linked recessive gene on the short arm of chromosome 4

Your answer is incorrect.

A single autosomal dominant gene on the short arm of chromosome 4 transmits Huntington's disease. There is Autosomal dominant inheritance with 100% penetrance; thus 50% of a patient's offspring will be affected. The specific genetic defect has been identified as an expansion of the trinucleotide sequence CAG between 37 and 120 repeats on chromosome 4.

The correct answer is: Autosomal dominant gene on the short arm of chromosome 4

Question **48**

Not answered

Marked out of 1

During pregnancy, where continued use of Valproate is deemed essential, what is the maximum dose recommended?

Select one:

- ☐ 800 mg
- ☐ 200 mg
- ☐ 1000 mg
- ☐ 400 mg
- ☐ 600 mg

Your answer is incorrect.

Where continued use of Valproate or carbamazepine is deemed essential, low dose monotherapy is strongly recommended, as the teratogenic effect is probably dose related. NICE recommends that the dose of Valproate should be limited to 1000 mg a day. Ref: Maudsley prescribing guidelines, 10th edition; 351

The correct answer is: 1000 mg

Question 49

Not answered

Marked out of 1

When used as mood stabilizers, what is the minimum period suggested to withdraw anticonvulsant drugs?

Select one:

- ☐ 3 months
- ☐ 1 month
- ☐ 1 week
- ☐ 1 year
- ☐ 6 months

Your answer is incorrect.

When used as mood stabilizers, it would be prudent to withdraw anticonvulsant drugs slowly. The optimum period of taper is unknown. A period of at least 1 month is suggested. When used for epilepsy, the dose should be gradually tapered over a period of 6 months. If the patient is taking more than one anticonvulsant, each drug should be withdrawn sequentially. Ref: Maudsley prescribing guidelines, 10th edition; 343

The correct answer is: 1 month



Question **50**

Not answered

Marked out of 1

What is the lifetime risk of suicide in Huntington's disease?

Select one:

- ☐ 0.05
- ☐ 20-25%
- ☐ 0.01
- ☐ 10-15%
- ☐ 0.02

Your answer is incorrect.

The suicide rate for patients with Huntington's disease is 4-6 times higher than that for the general population, being as high as 12.7%, and increasing further in those over the age of 50. Suicide is reported to be the 3rd common cause of death after pneumonia and cardiovascular disease in patients with Huntington's disease.

The correct answer is: 10-15%



Question **51**

Not answered

Marked out of 1

What is the most common behavioural change observed in patients with Huntington's disease?

Select one:

- ☐ Disinhibition
- ☐ Aggression
- ☐ Agitation
- ☐ Poor self care
- ☐ Lack of initiative

Your answer is incorrect.

The high prevalence of behavioral problems in patients with Huntington's disease was first recorded by Craufurd et al. in 2001. The most common behavioral change observed in patients with Huntington's disease is lack of initiative followed by poor judgement, blunting of affect, poor self-care, self-centeredness and inflexibility. Irritability, impulsivity and verbal outbursts are also reported ranging from 15 to 60% of cases. Ref: Craufurd D Thompson JC Snowden (2001); behavioral change in Huntington's disease. Neuropsychiatry and behavioral neurology; 14: 219-226

The correct answer is: Lack of initiative

Question 52

Not answered

Marked out of 1

Which of the following feature is associated more with Huntington's rather than Alzheimer's dementia?

Select one:

- ☐ Sub cortical type of dementia
- ☐ Language function significantly affected
- ☐ Impairment in executive function correlates with general cognitive decline
- ☐ Semantic memory significantly affected
- ☐ Visuospatial function significantly affected

Your answer is incorrect.

Sub cortical type of dementia is seen in Huntington's and cortical type is seen in Alzheimer's type. All the other features listed above are more associated with Alzheimer's type. Other features of Huntington's include gradual onset, early and significant impairment in executive function, disproportionate to other cognitive domains, language function relatively preserved until late, Visuospatial function and semantic memory relatively preserved until late. The predominant cognitive impairment in Huntington's disease is characterized by impaired executive function.

The correct answer is: Sub cortical type of dementia



Question **53**

Not answered

Marked out of 1

Choose an accurate statement with regard to depressive disorders in terminal illness

Select one:

- ☐ It may respond to amphetamines.
- ☐ It is generally diagnosed easily
- ☐ Presence of somatic symptoms strongly favour the diagnosis
- ☐ It does not respond to antidepressants
- ☐ Depression is uncommon in palliative care settings

Your answer is incorrect.

SSRIs do not have much evidence but are widely used in depressed patients receiving palliative care. Low dose amitriptyline (avoid in delirium risk; useful if neuropathic pain), lofepramine and rapid-acting psychostimulant such as dexamphetamine or methylphenidate (in a patient who has only weeks of life) are other useful options.

The correct answer is: It may respond to amphetamines.



Question **54**

Not answered

Marked out of 1

Which of the following tests would be required to make a diagnosis of Huntington's disease in a patient without a family history?

Select one:

- ☐ Electromyography
- ☐ Immunological tests
- ☐ Sleep studies
- ☐ Gene testing
- ☐ EEG

Your answer is incorrect.

Sporadic appearance of new mutations is not uncommon in the Huntington disease gene. Hence a genetic test is advised even in those who present without family history.

The correct answer is: Gene testing



Question **55**

Not answered

Marked out of 1

Mr. P has treatment resistant schizophrenia and is currently on clozapine 600 mg /day. Which anticonvulsant should be used to prevent clozapine induced seizures?

Select one:

- ☐ Lamotrigine
- ☐ Sodium Valproate
- ☐ Vigabatrin
- ☐ Carbamazepine
- ☐ Gabapentin

Your answer is incorrect.

Sodium Valproate is the anticonvulsant of choice as it has a lower incidence of leucopenia than carbamazepine.

The correct answer is: Sodium Valproate



Question 56

Not answered

Marked out of 1

Which of the following statements about periodic limb movement disorders (PLMD) is incorrect?

Select one:

- ☐ Its diagnosis requires polysomnographic documentation of an increased number of PLMS in association with significant disruption of sleep architecture and symptomatology.
- ☐ PLMS are a common phenomenon in otherwise healthy individuals.
- ☐ PLMD is an awake phenomenon
- ☐ These movements can cause clinical sleep disturbance expressed by insomnia or excessive daytime sleepiness.
- ☐ It is characterized by periodic episodes of repetitive and stereotyped limb movements that occur during sleep.

Your answer is incorrect.

PLMD is characterized by periodic episodes of repetitive and stereotyped limb movements that occur during sleep. These movements can cause clinical sleep disturbance expressed by insomnia or excessive daytime sleepiness. In contrast to RLS, PLMD is not an awake phenomenon but an asleep phenomenon, and its diagnosis requires polysomnographic documentation of an increased number of PLMS in association with significant disruption of sleep architecture and symptomatology. Polysomnographic diagnostic criteria for PLMS1 are based on electromyography (EMG) of the right and left anterior tibialis muscles. The movements may or may not be followed by arousal or awakening as determined by the polysomnogram. PLMS are a common phenomenon in otherwise healthy individuals. They also become more prevalent with increasing age (occurring in up to 34% of people over 60 years of age).

The correct answer is: PLMD is an awake phenomenon

Question **57**

Not answered

Marked out of 1

Many patients with Chronic Fatigue Syndrome will meet the criteria for another psychiatric diagnosis, which is most commonly;

Select one:

- ☐ Hypochondriasis
- ☐ Somatisation
- ☐ Anxiety disorder
- ☐ Conversion disorder
- ☐ Major depression

Your answer is incorrect.

Many patients with CFS will meet the criteria for another psychiatric diagnosis, most commonly major depression. Arguments exist for calling CFS a form of atypical depression; however this is untenable in a significant minority of cases. Not only do patients resist a 'psychiatric' diagnosis, but many do not have anhedonia or pervasively low mood, and attribute any mood disturbance to the restriction on their activities caused by their illness. Nonetheless, treatment of co morbid depressive or anxiety symptoms can produce clinical improvement. There is a high rate of depression in patient with CFS. 23% of CFS patient have current major depression and 50%- 75% have life time history of major depression (Wessley et .al, 1998). Ref: Oxford handbook of psychiatry-753

The correct answer is: Major depression

Question **58**

Not answered

Marked out of 1

A 75-year-old man with Parkinsons disease develops psychosis unrelated to levodopa use. Choose the best antipsychotic:

Select one:

- ☐ Olanzapine
- ☐ Haloperidol
- ☐ Risperidone
- ☐ Aripiprazole
- ☐ Quetiapine

Your answer is incorrect.

Eight open-label studies have assessed quetiapine use in 191 patients with psychosis and Parkinson's disease, with improvement in psychosis recorded in 152 (80%). In addition to the open-label studies, there have been two single-blind, randomized trials comparing quetiapine and clozapine. These studies suggest that quetiapine has similar efficacy to clozapine in controlling psychosis.

The correct answer is: Quetiapine



Question **59**

Not answered

Marked out of 1

Which of the following factors have been implicated in the generation of periodic limb movement disorders?

Select one:

- ☐ Iron deficiency
- ☐ Vitamin B 12 deficiency
- ☐ Copper deficiency
- ☐ Parathyroid deficiency
- ☐ Thyroid deficiency

Your answer is incorrect.

Dopaminergic impairment and iron deficiency have been implicated in the generation of PLMS, similar to restless leg syndrome.

The correct answer is: Iron deficiency



Question **60**

Not answered

Marked out of 1

Mrs.Z has a history of bipolar depression and is on treatment with mood stabilizer throughout her pregnancy. She gave birth to a male child with Cleft palate. The most likely offending drug is;

Select one:

- ☐ Vigabatrin
- ☐ Carbamazepine
- ☐ Sodium Valproate
- ☐ Gabapentin
- ☐ Lamotrigine

Your answer is incorrect.

Data for Lamotrigine suggests a low risk of foetal malformation when used as monotherapy, although a substantially increased risk of cleft palate has been reported. Clearance of Lamotrigine seems to increase radically during pregnancy. NICE suggest that Lamotrigine should not be routinely prescribed in pregnancy.

The correct answer is: Lamotrigine



Question **61**

Not answered

Marked out of 1

The core symptom of neurasthenia is

Select one:

- ☐ Headaches
- ☐ Pain
- ☐ Cognitive difficulties
- ☐ Fatigue
- ☐ Aching joints

Your answer is incorrect.

Chronic fatigue syndrome (CFS) is the current preferred name for a clinical syndrome whose central feature is severe fatigue, unrelated to exertion and unrelieved by rest (neurasthenia). Fatigue is a symptom characterised by a subjective feeling of lethargy, lack of energy, exhaustion, and a feeling of increased effort to do anything. Patients also complain of aching muscles, poor unrefreshing sleep, aching joints, and headaches. They may date the symptoms very precisely to an episode of viral infection with sore throat, fever, and tender lymph nodes. CFS is currently preferred to the names myalgic encephalomyelitis (ME) and post-viral fatigue syndrome as it doesn't imply knowledge of underlying pathology or aetiology. Ref: Oxford handbook of psychiatry-754.

The correct answer is: Fatigue

Question **62**

Not answered

Marked out of 1

Mr. Q suffers from epilepsy and psychosis with paranoid delusions and auditory hallucination. He is worried about metabolic side effects of newer antipsychotics and would like to take an older antipsychotic medication. The drug that should be avoided in his case would be;

Select one:

- ☐ Chlorpromazine
- ☐ Haloperidol
- ☐ Trifluoperazine
- ☐ Sulpiride
- ☐ All of the listed options

Your answer is incorrect.

Chlorpromazine is the most epileptogenic of the older drugs and ideally should be avoided completely. Sulpiride, Haloperidol and Trifluoperazine have a low pro-convulsive effect and are a good choice for epileptic patients. Ref: Maudsley prescribing guidelines, 10th edition; 337

The correct answer is: Chlorpromazine



[Refund Policy](#) | [Terms & Conditions](#)

